

NOREON HEALTH

Noreon Pilot, made easy

Recruitment to continuous cardiovascular risk
control

EASY-READ INTERACTIVE GUIDE

A plain-language companion to the Pilot operating blueprint (with the Pilot vs Control design). It does not replace the full document.

How this guide helps

- 1 **One idea per page.** Most pages take under a minute.
- 2 **Tap anything that looks like a button** to jump around.
- 3 **Tick boxes as you go.** They are real checkboxes.
- 4 **Stop any time.** Every page makes sense on its own.

Start: 1-minute how-to ►

Menu

How to use this guide (60 seconds)

1 Tap the buttons

Back, Menu and Next sit at the bottom of every page. On stage pages, tap a numbered dot to jump to another stage.

2 Start with “In one line”

Each page opens with the one-sentence version. Short on time? Read only that.

3 Tick as you go

The boxes are real checkboxes. In Adobe Acrobat or Preview, save the file to keep your ticks. Some browsers will not keep them.

4 Pause whenever you like

Each part ends with a “Good place to pause” tag. Come back through the Menu.

Colour key

Teal: Pilot group, patients

Amber: Control group, warnings

Navy: navigation, build notes

Grey: decisions

Colour is never the only clue: every colour also comes with a label.



Try it: tick this box

Menu: pick what you need

A

Start here

Big picture, what changed, the journey, Pilot vs Control.

about 6 min · 8 pages

B

The 9 stages

What happens at each step, who does it, and when it is done.

about 6 min · 9 pages

C

Rules & safety

The non-negotiables, who decides what, and how alerts work.

about 2 min · 3 pages

D

Patients & home

What patients and caregivers see, Zoe, and devices at home.

about 3 min · 4 pages

E

Build notes

Identity, data, screens, statuses, tasks and launch scope.

about 6 min · 8 pages

F

Decisions to close

53 open questions, each with a proposed default.

about 12 min · 17 pages

G

Measure & check

Success, metrics, a short quiz, glossary and notes.

about 6 min · 8 pages

Quick jumps

[Which part is mine?](#)[Quiz](#)[Glossary](#)[My notes](#)

Tip: you do not have to read in order. Jump to your role on “Which part is mine?”.

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START HERE

Which part is mine?

Tap a button to jump straight to what you need.

Front desk / admin

Stage 1: Recruitment

Stage 3: Onboarding

Pilot coordinator / medical lead

Stage 2: Assignment

Pilot vs Control

Decisions

Nurse

Stage 4: Measurements

Lab / diagnostic staff

Stage 5: Tests

Doctor (GP or Noreon)

Stage 6: Initial review

Stage 8: Results

Who decides what

Device coordinator

Stage 7: Devices

Devices at home

Product / engineering

Identity and QR

Data objects

Screens

Legal / ops / partners

Decisions to close

Rules

Flags in the draft

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The pilot in numbers

IN ONE LINE

A **controlled comparison** in primary care, with human supervision of every clinical decision. It is not a one-day screening event.

6 months

pilot length

3-4 sites

primary-care polyclinics

1,000-1,500

people assessed

300-500

actively managed patients

Hypertension

primary clinical pathway

Full human supervision

of clinical decisions

Planning assumptions from the source proposal. Made for: polyclinics · Noreon clinical staff · product and engineering.

What changed in this version

IN ONE LINE

A new **Stage 2** splits eligible people into **Pilot** and **Control**, so Noreon's effect can be compared with usual care.

WHAT IS NEW

- New **Stage 2**: eligibility selection and cohort assignment.
- Eligible people go to the **active Noreon Pilot** or a **Control benchmark**: usual care with matched outcome monitoring.
- Stages 3-9 (onboarding to home management) now apply to the **Pilot group**. Devices go to eligible Pilot participants.
- Recruitment now ends with a **selection visit**.
- New screen, objects and measures: cohort assignment, CohortAssignment, BenchmarkCheckpoint, cohort balance.

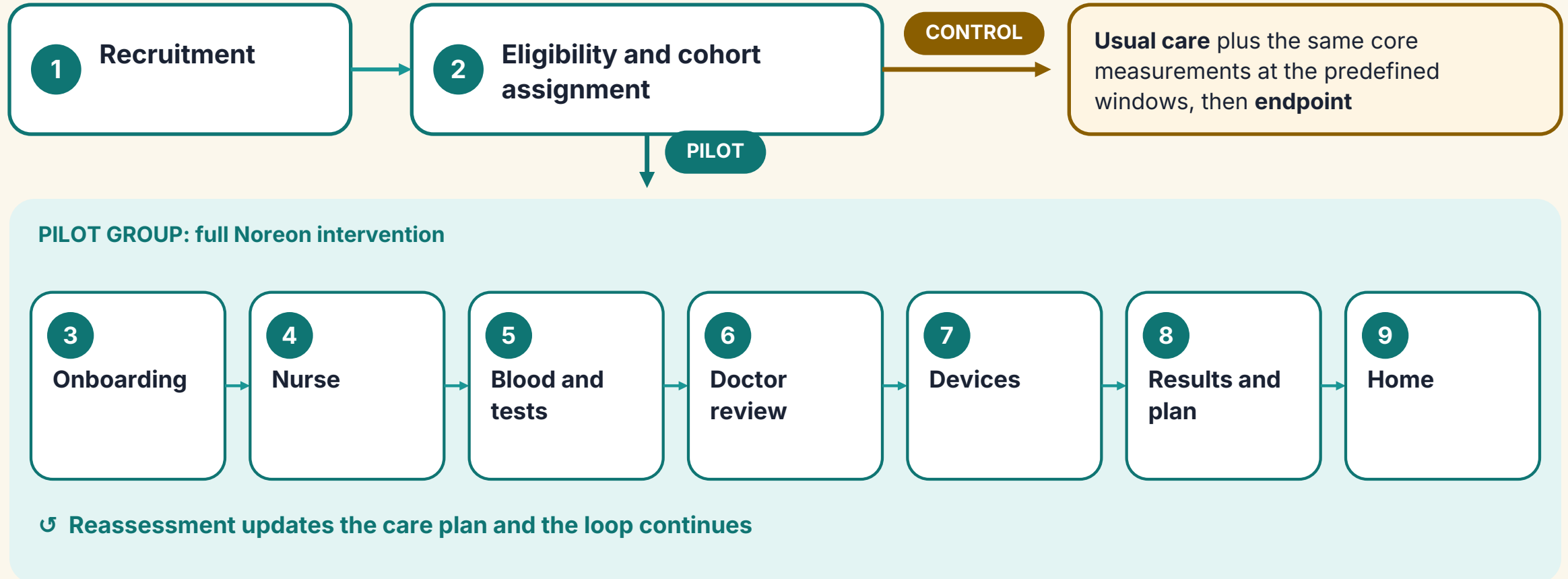
STAGE NUMBERS MOVED

Onboarding	2 →	3
Nurse measurements	3 →	4
Blood and tests	4 →	5
Initial doctor review	5 →	6
Device handover	6 →	7
Results and approval	7 →	8
Home management	8 →	9

START HERE

The journey on one page

Every box is a button. Blue is the Pilot group, amber is the Control group.



Nothing in the Pilot lane starts until Stage 2 has assigned the cohort.

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Pilot vs Control at a glance

	PILOT	CONTROL
Care	Noreon journey: clinician-approved plan, Zoe coaching, active follow-up.	Usual polyclinic care.
Devices and app	Connected BP monitor and pillbox if eligible. Full Noreon app.	No devices. At most a minimal control-mode app: consent, scheduling, questionnaires.
Measured	Common baseline, then the same core outcomes at the predefined windows.	Common baseline, then the same core outcomes at the same windows.
Safety	Red-flag routing, alerts and clinician review.	Normal care. Clinically important findings are not ignored "for study purity".
Tracked in	Station queues and the clinical-review queue.	A benchmark queue: baseline done, next measurement due, follow-up, withdrawal, crossover.

Crossover: if a Control patient starts a Noreon intervention, it is recorded explicitly. The assigned cohort never changes silently. **Ratio:** set in the analysis plan. 1:1 is the cleanest.

Stage 1: Recruitment

Owner: Polyclinic administrative staff

IN ONE LINE

Call patients on participating GPs' lists, **invite them to a selection visit**, and book it.

STEPS

- 1 **Open the queue.** One candidate at a time: name, phone, GP, site, reason, past attempts, free slots.
- 2 **Call.** Explain that their GP/polyclinic is in a cardiovascular risk-control program and invite them.
- 3 **Record the outcome.** Pick 1 of 9 (e.g. booked, call back later, declined, do-not-contact).
- 4 **Book the visit.** The patient gets a confirmation with site, time and any preparation.
- 5 **Status becomes SCHEDULED.** The site dashboard shows who is expected.

DONE WHEN

- ☐ Every call has an outcome
- ☐ Declined and unreachable leave the queue
- ☐ Visit booked (SCHEDULED)

WATCH OUT

Keep it minimal

Callers see only what they need to make the call: no diagnoses, no lab data. After a clear "no", stop calling.

Source §2.1 · Screen: Recruitment and call queue (§7.1)

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Stage 2: Eligibility and cohort assignment

Owner: Pilot coordinator + medical lead (statistical plan fixed before enrollment)

IN ONE LINE

Same baseline for all, then **assign Pilot or Control** before any intervention.

STEPS

- 1 Evaluation consent.** Patient agrees, including possible assignment to Control.
- 2 Common baseline.** Same variables, same way, before any intervention.
- 3 Eligibility gate.** Approved criteria. Record ELIGIBLE or INELIGIBLE, version, reason.
- 4 Assign the cohort.** PILOT or CONTROL by the pre-approved method (randomized preferred).
- 5 Pilot branch.** Noreon onboarding, tests, devices, Zoe, active follow-up.
- 6 Control branch.** Usual care plus benchmark measurements at set windows.

DONE WHEN

- ☐ Consent + common baseline done
- ☐ Eligibility + protocol version logged
- ☐ Cohort assigned, locked, logged

WATCH OUT

Never choose the group

Putting sicker or more motivated people in Pilot ruins the benchmark. Switching needs an authorized, documented reason.

Source §2.2 · Screen: Eligibility and cohort-assignment screen (§7.2)

Stage 3: Pilot onboarding (Pilot group only)

Owner: Front desk / pilot admin + patient

IN ONE LINE

Set up the **Pilot** patient: app, permissions, history and health questionnaire, ready for the nurse.

STEPS

- 1 Confirm identity.** Two identifiers. Start the pilot encounter.
- 2 Activate the app.** Give the Noreon QR or link. Help if needed.
- 3 Consent.** The system logs the version, time and each scope granted.
- 4 Add history.** Staff upload relevant ArMed data. The patient can scan or upload older documents.
- 5 Anamnesis.** The patient answers in small batches. Staff see progress, not unneeded answers.
- 6 Draft recommendations.** Tests and measurements with reasons. They stay DRAFT until approved.
- 7 Station QR.** The app shows a time-limited QR for the rest of the visit.

DONE WHEN

- ☐ Identity confirmed
- ☐ Required consent is valid
- ☐ Baseline tasks can be created

WATCH OUT

No automatic ArMed link

In this pilot, staff move ArMed data by hand, after consent. Every import and export is logged: source, operator, time.

Source §2.3 · Screen: Front-desk onboarding screen (§7.3)

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Stage 4: Nurse measurements

Owner: Nurse

IN ONE LINE

Scan the QR, **confirm who they are**, then record height, weight, waist, BP and pulse.

STEPS

- 1 **Scan** the station QR in the authenticated staff app (phone or tablet).
- 2 **Confirm the person.** First name, surname, year of birth (photo only if governance permits).
- 3 **Follow the checklist.** Only the measurements due for this patient appear.
- 4 **Validation.** Odd value? The app asks for a re-check or a comment. No silent blocking or fixing.
- 5 **Save with provenance.** Patient, encounter, operator, device or manual, time, units, protocol version.
- 6 **Station complete.** "All required measurements captured", or a clear list of what remains.

DONE WHEN

- ☐ All required values captured
- ☐ Anything missing is listed
- ☐ Each value saved with provenance

WATCH OUT

See the person first

After a scan, the nurse must see and confirm who the patient is before any editable form. The number of BP readings follows the approved protocol.

Source §2.4 · Screen: Nurse measurement capture (§7.4)

Stage 5: Blood and diagnostic tests

Owner: Lab collector + diagnostic technician/nurse

IN ONE LINE

Link the **specimen barcode** or **device session** to the patient so results never get mixed up.

STEPS

- 1 Blood: scan** the patient QR. The lab screen shows the ordered tests.
- 2 Blood: link** the lab's specimen barcode (scan or create). Check tubes and order status.
- 3 Blood: finish** by marking COLLECTION_COMPLETE.
- 4 ECG/EEG: scan** the patient QR and start the device session.
- 5 ECG/EEG: link.** Noreon stores the session ID. The result matches to it automatically.
- 6 Scan failed?** Two-identifier manual lookup. A reason is required and it is audit-logged.

DONE WHEN

- ☐ Barcode linked to patient + visit
- ☐ Device session ID linked
- ☐ Collection marked complete

WATCH OUT

QR = person. Barcode = sample.

Never join results by typing a name. Still open: ECG or EEG? A 12-lead ECG is the proposed default for hypertension.

Source §2.5 · Screen: Lab collection (§7.5) and ECG/EEG station (§7.6)

Stage 6: Initial doctor review

Owner: Participating GP / designated onsite doctor

IN ONE LINE

Give **safe preliminary instructions** and set the home BP plan while some results are pending.

STEPS

- 1 Read the doctor screen.** Anamnesis summary, records, measurements, tests done and pending, medications, red flags.
- 2 Confirm the working problem list.**
- 3 Give preliminary instructions** that are safe before the pending results arrive.
- 4 Verify current medication use.**
- 5 Set the initial home BP-measurement plan.**
- 6 Say what happens while results are pending.** Record any immediate escalation.

DONE WHEN

- ☐ Plan saved as PRELIMINARY
- ☐ Pending results listed
- ☐ Any escalation recorded

WATCH OUT

Decisions, not data entry

The doctor should not re-type what the patient, nurse, lab or devices already captured. The screen highlights gaps, contradictions and decisions.

Source §2.6 · Screen: Initial doctor screen (§7.7)

Stage 7: Device handover and discharge

Owner: Admin / device coordinator

IN ONE LINE

Hand over the BP monitor (and pillbox if eligible), **prove it sends data**, then discharge.

STEPS

- 1 **Scan the patient QR.** Confirm the person.
- 2 **Assign equipment.** Scan the BP monitor serial/QR and, where applicable, the pillbox.
- 3 **Pair.** Guide the patient through Bluetooth/app pairing and permissions.
- 4 **Test.** No completion until Noreon confirms data arrived, or a documented non-connected fallback is chosen.
- 5 **Explain the next 24-48 hours.** Current home actions, what is still processing, how they will know the reviewed plan is ready.
- 6 **Custody record.** Device, accessories, condition, ownership/loan terms, return policy acknowledged.

DISCHARGE GATE: ALL FOUR "YES"

- ☐ Is the app active?
- ☐ Are devices connected, or a fallback defined?
- ☐ Does the patient know what to do next?
- ☐ Is there a named route for help?

WATCH OUT

Pairing failed = not issued

Create a support task instead of marking the device issued. Only Pilot patients receive devices.

Source §2.7 · Screen: Device provisioning screen (§7.8)

Stage 8: Results, approval and ArMed update

Owner: Noreon doctor + GP + polyclinic staff

IN ONE LINE

AI drafts, **a clinician approves**, the plan is published, and ArMed is updated by hand.

STEPS

- 1 Data-quality check.** Identity, expected tests, units, duplicates.
- 2 Draft synthesis.** What was found, what is missing, proposed actions.
- 3 Noreon clinical review.** Approve, modify, ask, or route to GP/specialist.
- 4 GP routing.** FYI for routine. ACTION REQUIRED for physician decisions.
- 5 Publish.** Only an approved plan version reaches the patient.
- 6 Manual ArMed update.** Staff upload results. Noreon logs who and when.
- 7 Notify the patient.** Plain-language overview first, then drill-down.

DONE WHEN

- ☐ Only the approved plan published
- ☐ Approver and time recorded
- ☐ ArMed update task completed

WATCH OUT

AI drafts, clinicians approve

Nothing reaches the patient dashboard until a clinician approves that plan version.

Source §2.8 · Screen: Clinical-review workspace (§7.9), GP inbox (§7.10)

Stage 9: Home management and follow-through

Owner: Patient + Zoe + care team + caregiver (if enabled)

IN ONE LINE

Daily life, organised by **actions**, not by medical documents. The loop never really ends: reassessment updates the plan.

Medication

Schedule exactly as approved, taken or not, refill tasks, pillbox data.

Blood pressure

When to measure, technique help, connected readings, trend, on track?

Physical activity

A realistic goal from the approved plan. Adapts only inside safe limits.

Nutrition

Only the active goals for this plan. No generic content feed.

Sleep

Shown only when sleep matters clinically or for behaviour.

Smoking and alcohol

Only when relevant. Goal-based and non-judgmental.

Tests and appointments

Next test, visit, GP call or cardiology consult: date, preparation, rescheduling.

Questions and support

Ask the GP/medical team, or book a cardiology video consult when allowed.

🔗 **Next tests and visits come from the approved plan. Repeated rescheduling is a care gap, not just a calendar event.**

Source §2.9 · Screen: Patient app (§7.11)

Eight rules that never bend

Tick each one once you could explain it to a colleague.

1 Humans approve

AI drafts. Clinical actions stay blocked until the responsible clinician approves.

☐ I can explain this

2 No made-up thresholds

Red flags, BP reading counts and response times come from the medical team's versioned rules.

☐ I can explain this

3 QR holds no secrets

Opaque, short-lived token. Scanning a QR is not logging in.

☐ I can explain this

4 See the person first

Confirm identity before any editable form appears.

☐ I can explain this

5 Assignment is not a choice

Pilot or Control by the pre-approved method only. No staff discretion.

☐ I can explain this

6 Zoe stays in the plan

Explains and supports. No medical advice, no medication or dose changes.

☐ I can explain this

7 Safety beats nudging

Clinical safety rules always override behaviour optimisation.

☐ I can explain this

8 Log everything

Every scan, edit, rematch, approval, export and device assignment is audited.

☐ I can explain this

Who decides what

IN ONE LINE

Clinicians sort the work into **four queues** so nothing is one big pile. AI drafts and prepares. A named clinician decides.

FYI

Routine education, adherence and monitoring inside an approved plan. No forced click unless the GP wants detail.

REVIEW REQUESTED

Someone has asked for a clinician's review. A separate state, so the inbox is not one pile.

ACTION REQUIRED

Decisions that stay with the treating physician: medication changes, new diagnoses, changed targets, referrals.

URGENT

Red-flag events. Every alert needs an owner and a due-by time (see the alerts page).

No auto-changes

Noreon cannot change a medication or dose from home data. It flags the need, prepares evidence and a proposal, and routes it to the clinician.

Disagreement

If the Noreon doctor and the GP disagree, the disputed part stays pending until the accountable clinician decides. Both proposals are recorded.

One name per plan

Each plan version has one accountable approver. Every approved action is tied to a named clinician and a plan version.

Cardiology referral has 3 separate reasons: protocol-based, clinician-requested, patient-requested. The medical team sets the criteria.

Alerts and the safety net

IN ONE LINE

The **medical team** defines what counts as urgent. The product never invents a threshold.

1. What triggers an alert?

- Symptoms
- Measurement thresholds
- Device events
- Combinations of data

All from a **versioned red-flag and escalation protocol** provided by the medical team.

2. Who owns it?

A **named role** and a **visible queue** at each site, backed by Noreon clinical safety oversight.

Every alert records:

- Owner
- Created time and severity
- Due-by time
- Outcome and escalation history

3. Outside clinic hours

Define clear hours and an out-of-hours message. The app must tell apart:

"The team will review this"

"Seek urgent care now", as the approved protocol defines

Response times (SLAs) are stored per alert type and are not hard-coded until the medical team approves them.

What the patient sees

THE HOME SCREEN ANSWERS 3 QUESTIONS

- 1 What do I need to do **today**?
- 2 What is **coming next**?
- 3 Is anything **waiting for the medical team**?

HOME SECTIONS

Today's actions Blood pressure Medicines Next appointment

Next test Activity / nutrition / sleep (when active)

Messages from the medical team Progress

RESULTS: REVIEWED ONES ONLY. EACH ONE SHOWS...

What it means (plain language)



Why it matters



What the plan asks next



Is a clinician action pending?

Wording

Calm, plain language. No raw labels like "abnormal" or "high-risk alert" without an explanation and a next step.

Appointments

Confirm, ask to reschedule, or cancel. The app must show which visits are flexible and which are clinically constrained.

The caregiver account

IN ONE LINE

A caregiver sees **only what the patient chooses to share**. They help, but they cannot change the care plan.

CAN

- View selected daily tasks
- See if medication and BP actions were done
- See upcoming appointments
- Receive selected reminders
- Help with device connectivity
- Message support, where permitted

CANNOT

- Change medicines
- Change clinical targets
- Reschedule restricted appointments without authority
- View medical details the patient did not share
- Add another caregiver without the patient's authorization

THE PATIENT STAYS IN CONTROL

Caregiver has their own account

Shared per category

Revoked instantly, and logged

Reminders only if the patient opts in

Zoe and the behaviour loop



- 1 Observe** Plan needs, medication events, BP data, appointments, tests, activity, self-report.
- 2 Understand** Why is it (not) happening? Capability, opportunity, motivation. Non-adherence is not low motivation.
- 3 Prioritize** The most important, achievable next action. Not five reminders for five tasks.
- 4 Intervene** An approved technique: instruction, planning, cues, feedback, problem solving, social support, habits.
- 5 Measure** Did the action happen in its window? Did the health indicator move?
- 6 Learn** Which timing, channel and framing work for this person.
- 7 Adapt** Fewer prompts as habits form. Silence is an allowed action.

Safety first. Clinical safety rules override behaviour optimisation. Zoe cannot change medication, dose, diagnostic strategy or clinician-set thresholds. A caregiver is an option only if the patient allowed it, never an automatic escalation.

Devices at home

Pilot group only. Control patients receive no connected devices.

1 Who gets what

BP monitor: every actively managed Pilot participant if supply allows, otherwise those who need structured home BP. Record why. Pillbox: a defined rule, not everyone.

2 Prove it works

A test reading and a green "Connected and receiving data" state are required before handover. If pairing fails, create a support task. Do not mark it issued.

3 Who owns it

Put ownership in the device agreement. Loans are easiest to track. Define deposit, damage, loss, replacement and return.

4 Linking

Admin scans the patient QR, then the device serial/QR. This creates a DeviceAssignment: type, serial, patient, staff, date, status, test result.

5 No smartphone?

Plan a pathway before launch: caregiver-assisted, clinic-synced, phone-call workflow, or exclusion from connected-device sub-studies. The main pathway must not depend on it.

6 Disconnected at home?

Detect missing transmissions. Tell non-use from technical failure. Guide self-reconnection first, then create a human support task.

How identity works (QR)

IN ONE LINE

The QR identifies the **person and visit**. Barcodes and session IDs identify the **sample or test**. Names are never used to join data.

1 Patient app

Shows an opaque, signed, time-limited QR token. It reveals nothing on its own.

2 Staff scan

Inside an authenticated staff app with role-based access. A QR scan is not authentication.

3 Station session

Short-lived: patient, encounter, staff, role, site, station type, created and expiry times.

4 Positive ID

Staff see minimal identity details and confirm before data entry. Never straight into an editable form.

5 Saved and audited

Records link to one stable person ID, not a name. Every scan, edit, rematch, approval, export and assignment is logged.

CROSSWALKS: LINKING OUTSIDE SYSTEMS

NoreonOrderID



LabOrderID



SpecimenBarcode

NoreonDiagnosticOrderID



DeviceSessionID

Never overwrite the partner's original identifiers. If a scan fails: two-identifier manual lookup, a reason, and an audit entry.

The data objects (21 of them)

PEOPLE AND PROGRAMME

Person

one stable ID + identity data

ProgramEnrollment

pilot, site, GP, source, consent state, dates

CohortAssignment

eligibility, protocol version, method, cohort, strata, time, override reason

BenchmarkCheckpoint

cohort, checkpoint, due window, outcome set, values

Consent

version, scope, accepted / revoked time

CaregiverLink

caregiver, patient, scope, status

VISITS AND CLINICAL

Encounter

baseline, follow-up or telemedicine

Anamnesis

versioned answers + provenance

ClinicalOrder

measurement, lab or diagnostic order

Measurement

value, units, source, time, operator

Specimen

Noreon order ID + lab IDs + states

DiagnosticSession

order, device session ID, files

ClinicalResult

structured result + original file

CarePlanVersion

targets, meds, tests, goals, approver

Appointment

type, provider, time, rescheduling rules

BEHAVIOUR, SAFETY, DEVICES

BehaviorGoal

behaviour, target, barrier hypothesis

InterventionEvent

technique, channel, response

Alert

trigger, severity, owner, due-by

AuditEvent

actor, action, object, before/after, time

Device

type, model, serial, inventory state

DeviceAssignment

person, device, issue/return, pairing

The 15 screens

STAFF STATIONS

- 7.1 Recruitment queue.** Call, record outcome, book.
- 7.2 Eligibility and cohort.** Split Pilot and Control. Benchmark queue.
- 7.3 Front-desk onboarding.** Check-in, consent, journey bar.
- 7.4 Nurse capture.** Scan, confirm, measure, one-handed.
- 7.5 Lab collection.** Specimen linking, exception queue.
- 7.6 ECG/EEG station.** Device session and result states.
- 7.8 Device provisioning.** Assign, pair, test, custody, inventory.

CLINICIANS

- 7.7 Initial doctor screen.** Decisions, not data entry.
- 7.9 Noreon review workspace.** Cases by urgency.
- 7.10 GP inbox.** Action required, FYI, follow-up, urgent.
- 7.13 Telemedicine.** Pre-call summary, notes, draft visit summary.

PATIENTS AND CARERS

- 7.11 Patient app.** Today's actions, results, plan, documents, Zoe.
- 7.12 Caregiver view.** Shared with you by the patient.

OVERSIGHT

- 7.14 Operations dashboard.** Funnel, queues, device success.
- 7.15 Safety and governance.** Alerts, deviations, audit export.

Statuses, part 1: before the visit

RECRUITMENT

IDENTIFIED → CONTACT_QUEUED → CONTACTED → SCHEDULED → SELECTION_PENDING

Ends: DECLINED · UNREACHED · DO_NOT_CONTACT · INELIGIBLE

SELECTION AND COHORT

SELECTION_PENDING → ELIGIBILITY_BASELINE_COMPLETE → ELIGIBLE → COHORT_ASSIGNED_PILOT or COHORT_ASSIGNED_CONTROL

Ends: INELIGIBLE · WITHDRAWN_BEFORE_ASSIGNMENT. The cohort is locked after allocation.

CONTROL BENCHMARK

COHORT_ASSIGNED_CONTROL → CONTROL_BASELINE_COMPLETE → CONTROL_MONITORING → CONTROL_FOLLOW_UP_DUE

CONTROL_FOLLOW_UP_COMPLETE → CONTROL_ENDPOINT_COMPLETE

Control stays in usual care. A crossover is recorded explicitly, never as a silent cohort change.

Statuses, part 2: Pilot, clinical, home, exit

PILOT ONSITE



CLINICAL



HOME MANAGEMENT



EXIT



Important: enrollment status and clinical-risk status are separate. A person never becomes “urgent” or “high risk” just because their enrollment status changed.

Every event needs an owner

IN ONE LINE

Every actionable event creates a **patient action**, **staff task**, **clinical review item** or **system event**. Never rely on a notification alone.

Missed BP measurement



The behaviour engine decides whether to intervene

Repeated missing BP + technical failure



A device-support task

New lab result



A clinical review event

Patient reports a medication concern



Clinical review, according to protocol

Appointment overdue



A patient nudge or admin outreach, depending on the barrier

Device disconnected



Self-help first, then a support task if unresolved

Caregiver invitation accepted



Permission update + optional caregiver onboarding tasks

SLA settings: target response times are stored per alert or task type. Do not hard-code them until the medical team approves.

Must have at launch

RECRUIT AND SELECT

- Recruitment / call queue
- Selection and eligibility workspace
- Versioned inclusion/exclusion criteria
- Cohort allocation + assignment lock
- Control benchmark roster + follow-up tracking
- Appointment scheduling

ONBOARD AND CAPTURE

- App activation
- Consent and versioning
- Anamnesis
- Patient scan/upload of prior documents
- Manual ArMed to Noreon and back
- QR identity layer
- Nurse capture
- Lab/specimen mapping
- Diagnostic-session mapping
- Initial doctor screen
- Device inventory and provisioning

REVIEW, PLAN, FOLLOW-UP

- Result ingestion
- Noreon clinical review
- GP FYI / action queue
- Versioned care plan
- Patient dashboard
- BP and medication-adherence flows
- Appointments
- Caregiver permissions
- Safety alerts
- Audit trail
- Pilot-versus-control analytics

Can wait, and must not wait

CAN BE PHASED IF NECESSARY

- Voice anamnesis
- Advanced automatic lab integration, if structured upload works safely
- Sophisticated receptivity modelling
- Broad lifestyle modules beyond the top-priority risk factors
- Contextual bandits / adaptive ML
- Automatic ArMed/EHR integration
- Automated clinician-note drafting beyond basic templates

DO NOT DEFER

- Identity safety
- Consent
- Provenance
- Clinical approval
- Device-assignment audit
- Red-flag routing
- Data-quality exceptions
- Task ownership

Decisions to close before build

IN ONE LINE

53 open questions, each with a proposed default. They are operating defaults, **not final clinical policy**. The medical, legal, operations and partner teams confirm or change them before the pilot configuration is frozen.

Who we recruit

§1.1 · 6 questions

Open

Pilot vs Control rules

§1.1 · 5 questions

Open

Consent and data

§1.2 · 6 questions

Open

Onsite tests

§1.3 · 8 questions

Open

Clinical responsibility

§1.4 · 7 questions

Open

Devices at home

§1.5 · 7 questions

Open

Behaviour and caregivers

§1.6 · 7 questions

Open

Safety and follow-up

§1.7 · 7 questions

Open

Tick "Decided" on each row as the answer is agreed. Save a copy of the PDF to keep your ticks.

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Who we recruit (1/2)

Source §1.1 · tick “Decided” when the answer is agreed

☐

Decided

Which people can staff contact?

Patients already attached to participating GPs, using the minimum info for a call list. The medical team defines versioned inclusion/exclusion criteria. Software never infers final eligibility from thin admin data.

☐

Decided

Who decides eligibility?

Two steps: admin pre-screening builds the pool. Final eligibility is confirmed at selection with the approved criteria and common baseline. The same rules apply to Pilot and Control.

☐

Decided

What may the caller see?

Only name, phone, GP, site, broad reason and appointment slots. No diagnoses or lab data unless the protocol requires them.

Who we recruit (2/2)

Source §1.1 · tick “Decided” when the answer is agreed

☐

Decided

What if the person declines?

Record a structured outcome with an optional short reason. No pressure. No repeat calls unless they ask.

☐

Decided

How many contact attempts?

A limited, configurable sequence (e.g. call, second attempt, one final follow-up). Operations and ethics/legal approve. The product enforces a cap and a do-not-contact state.

☐

Decided

Can family members be contacted?

Only if the person authorized that channel, or local rules and the protocol allow it. Caregiver enrollment normally comes after the patient consents.

Pilot vs Control rules (1/2)

Source §1.1 · tick "Decided" when the answer is agreed

☐

Decided

When does cohort assignment happen?

After evaluation consent and the minimum common baseline, but before any Noreon intervention, device, Zoe coaching or treatment-support workflow.

☐

Decided

How are patients assigned?

Same criteria for both groups. Randomized is preferred, stratified at least by site/GP and key baseline risk. Otherwise a prospectively defined matching method, documented before enrollment.

☐

Decided

What allocation ratio?

Set in the statistical analysis plan. 1:1 gives the cleanest benchmark. Use another ratio only with a documented reason.

Pilot vs Control rules (2/2)

Source §1.1 · tick "Decided" when the answer is agreed

☐

Decided

What does Control receive?

Usual care plus benchmark measurements at the same windows. No coaching, nudges, devices, pillbox or adherence support. Clinically significant findings still follow normal safety duties.

☐

Decided

Should Control use the Noreon app?

Avoid the full experience: the product itself changes behaviour. If needed, a minimal control mode (consent, scheduling, questionnaires), with no coaching or feedback.

Consent and data (1/2)

Source §1.2 · tick “Decided” when the answer is agreed

☐

Decided

What exactly is the patient consenting to?

Separate components: pilot participation, Noreon data processing, access to specified medical info, device connectivity, communications, optional caregiver sharing. Do not bundle caregiver access.

☐

Decided

How do Noreon and ArMed interact?

No automatic integration. After consent, staff extract ArMed data and upload it to Noreon, then upload results and the plan back. Every manual action is logged.

☐

Decided

Can patients add their own documents?

Yes: phone scan or PDF/image upload. Records keep their source and a visible review state. Staff can help.

Consent and data (2/2)

Source §1.2 · tick “Decided” when the answer is agreed

☐

Decided

Refuses record access but wants to join?

Allow it if the medical team considers it safe. Mark missing data clearly and confirm critical history manually. Never assume missing means normal.

☐

Decided

How does consent withdrawal work?

The patient can withdraw future participation and revoke caregiver access. Keep the legally required audit trail and separate withdrawal from deletion duties.

☐

Decided

Should the QR hold personal data?

No. An opaque, short-lived token resolved inside authenticated staff tools. A screenshot must reveal nothing.

Onsite tests (1/2)

Source §1.3 · tick “Decided” when the answer is agreed

☐

Decided

Anamnesis before measurements and tests?

Prefer it before the recommendation engine runs, but let the site continue. Show which recommendations are provisional.

☐

Decided

Which nurse measurements are mandatory?

The medical team approves the set. Support at least height, weight, waist, BP and pulse, plus repeats and technique metadata.

☐

Decided

How many clinic BP readings?

Do not hard-code. Build a configurable protocol: rest time, repeats, arm, cuff size, timing, reason for extra readings.

☐

Decided

ECG or EEG?

Confirm before development. A 12-lead ECG is the obvious default for hypertension. EEG needs its own indication and workflow. Never substitute silently.

Onsite tests (2/2)

Source §1.3 · tick "Decided" when the answer is agreed

☐

Decided

Which blood tests for everyone?

A mandatory baseline panel plus conditional orders from anamnesis and rules. Each order has an indication, priority, sample needs and source.

☐

Decided

May the engine order tests itself?

It may draft orders. Keep human approval unless governance pre-authorizes specific protocol-bound tests.

☐

Decided

Test cannot be done the same day?

Create a pending task with due date, booking state and owner. The patient sees the next action. Staff see overdue items.

☐

Decided

Lab or device cannot integrate by API?

Use a controlled bridge: QR/patient-code mapping plus structured upload. Keep source, time, original ID. Manual matching is an exception with two identifiers.

Clinical responsibility (1/2)

Source §1.4 · tick "Decided" when the answer is agreed

☐

Decided

Who approves the final post-results plan?

One accountable approver per plan version. The Noreon reviewer may approve the report if governance permits. The GP explicitly approves medication changes, new diagnoses and other treating-physician actions.

☐

Decided

Is the GP FYI or an approver?

Split the queue. Routine actions inside an approved plan are FYI. Medication changes, new diagnoses, changed targets and referral decisions are action-required.

☐

Decided

Can Noreon change a medication or dose?

No, unless a specific protocol and legal approval allow it. It can flag the need, prepare evidence and a proposal, and route it to the clinician.

☐

Decided

What does the patient see during review?

A calm status such as "Your results are with the medical team", plus the next step. No unreviewed AI interpretation as a final conclusion.

Clinical responsibility (2/2)

Source §1.4 · tick "Decided" when the answer is agreed

☐

Decided

Noreon doctor and GP disagree?

The plan, or the disputed part, stays pending until the accountable clinician resolves it. Record both proposals and the decision.

☐

Decided

When is cardiology mandatory?

The medical team defines versioned escalation criteria. Support protocol-based, clinician-requested and patient-requested referral as separate reasons.

☐

Decided

What must be versioned?

Every care plan, rule set, questionnaire, consent form, interpretation template and escalation threshold.

Devices at home (1/2)

Source §1.5 · tick “Decided” when the answer is agreed



Decided

Who gets a connected BP monitor?

Define eligibility explicitly. If supply permits, every actively managed Pilot participant. Otherwise prioritize those needing structured home BP, and record why.



Decided

Who gets a smart pillbox?

A defined rule, not everyone, unless the pilot tests universal use. Good candidates take prescribed daily medication where adherence matters.



Decided

Who owns the devices?

Make it explicit in the device agreement. Loans are easier to inventory, but define deposit, damage, loss, replacement and return.



Decided

How is a device linked to the patient?

Admin scans the patient QR, then the device serial/QR. The system creates a DeviceAssignment: type, serial, patient, staff, date, status, test result.

Devices at home (2/2)

Source §1.5 · tick “Decided” when the answer is agreed

☐

Decided

Does it send data before they leave?

Require a test reading and a green “Connected and receiving data” state. If pairing fails, create a support task. Do not mark it issued.

☐

Decided

No smartphone, or an incompatible one?

Define a non-smartphone pathway before launch: caregiver-assisted, clinic-synced, phone-call workflow, or exclusion from connected-device sub-studies.

☐

Decided

A device disconnects at home?

Detect missing transmissions, tell non-use from technical failure, guide self-reconnection, then create a human support task.

Behaviour and caregivers (1/2)

Source §1.6 · tick "Decided" when the answer is agreed

☐

Decided

Which behaviours are managed?

A small set tied to the plan: medication, BP measurement, test completion, appointments, top lifestyle risks. Activity, nutrition, sleep, smoking and alcohol only when relevant.

☐

Decided

Does every missed task trigger a reminder?

No. Weigh clinical importance, timing, past response, likely barrier and notification burden. Silence is an allowed action.

☐

Decided

Repeated non-adherence?

Diagnose the barrier before sending more messages. It may need education, a cue, problem solving, social support or a clinician conversation.

☐

Decided

Can Zoe give advice outside the plan?

No. Zoe explains the plan, supports action and collects symptoms and barriers. Medical decisions go through safety rules and the clinical team.

Behaviour and caregivers (2/2)

Source §1.6 · tick “Decided” when the answer is agreed

☐

Decided

What can a caregiver see?

Only what the patient shares. A mirror account cannot change the plan or see hidden data.

☐

Decided

Can a caregiver get reminders?

Yes, if the patient opts in to that type. Patient and caregiver notification preferences are separate.

☐

Decided

Can the patient revoke access instantly?

Yes. It takes effect immediately for future access and is logged.

Safety and follow-up (1/2)

Source §1.7 · tick "Decided" when the answer is agreed



Decided

Which events create urgent alerts?

The medical team provides a versioned red-flag protocol: symptoms, thresholds, device events, combinations. The product never invents thresholds.



Decided

Who watches alerts?

A named role and a visible queue per site, backed by Noreon clinical safety oversight. Each alert has an owner, time, severity, due-by, outcome and escalation history.



Decided

What happens outside clinic hours?

Define hours and an out-of-hours message. The app must separate "the team will review this" from "seek urgent care".



Decided

How are follow-up dates set?

The approved plan sets next-test and next-visit due dates. Patients reschedule within permitted windows. Clinically constrained visits need staff confirmation.

Safety and follow-up (2/2)

Source §1.7 · tick "Decided" when the answer is agreed

☐

Decided

Repeated rescheduling?

Treat it as a care-gap event. After a set number of delays, Zoe asks what is in the way and routes persistent barriers to staff.

☐

Decided

When does someone leave active management?

Define completion, withdrawal, transfer, loss-to-follow-up and safety exit separately. Keep outcomes and reasons.

☐

Decided

Evidence needed before more automation?

Safety first, outcomes at least as good as usual care, strong AI-clinician agreement, acceptable trust, and explicit governance approval.

What a successful pilot looks like

IN ONE LINE

Success is **not** “people downloaded the app”. It is a fair, safe comparison that shows Noreon’s **extra effect**.

- ☐ Recruitment produced a clearly defined **eligible population**.
- ☐ Eligible people were assigned to Pilot and Control by a **pre-specified method, before** intervention exposure.
- ☐ Both groups completed **comparable baseline and follow-up** measurements.
- ☐ The Pilot turned reviewed results into an **approved plan and daily actions**. Adherence and connected data were usable.
- ☐ Control stayed on **usual care**, without contamination.
- ☐ **Attrition and crossover** were documented.
- ☐ Noreon’s extra effect on BP and other agreed risk factors can be **compared credibly** with the benchmark, **without compromising safety**.

THREE PROOF AREAS

- 1 Better **clinical outcomes**
- 2 Greater **care-team capacity**
- 3 **Safety**

The pilot adds **flow-quality and behaviour measures** so failures can be diagnosed.

What we measure

1. Recruitment funnel

Identified → contacted → reached → invited → booked → arrived → consented → eligible → assigned (Pilot / Control) → baseline done → retained.

2. Cohort balance

Baseline characteristics of both groups, allocation counts, missing data, withdrawals, crossovers, contamination, follow-up completeness, timing gaps.

3. Clinical outcomes

Same in both groups: % reaching the physician-set BP target, mean change in systolic/diastolic BP, secondary risk-factor change.

4. Care-team capacity

Patients per physician, human minutes per patient per month, % routine interactions without physician time, % resolved without referral.

5. Engagement

Home BP data completeness, adherence, appointment/test completion, retention at 3 and 6 months, re-engagement, notification burden.

6. Operational quality

Onboarding time, nurse station time, lab/diagnostic match success, manual rematch rate, device pairing, uptime, review and publish latency, no-shows.

7. Safety

Serious adverse events, missed warning symptoms, overdue urgent reviews, AI-clinician agreement, AI messaging errors, deviations, identity-matching incidents.

Remember

The main comparison keeps each person's **original assigned cohort**. Crossovers and protocol deviations are reported separately.

Care-plan action completion is a Pilot-only process measure, not a Control outcome.

Quick check (1 of 2)

Pick one answer per question. Answers are on the next page. No score is kept.

1 When does cohort assignment happen?

- ☐ A. After the patient is onboarded to the Noreon app
- ☐ B. After consent and baseline, before any Noreon intervention
- ☐ C. After the first doctor review

2 What does the patient's station QR contain?

- ☐ A. Name and diagnosis
- ☐ B. Medical record number
- ☐ C. An opaque, short-lived token

3 Can Noreon change a medication dose from home data in this pilot?

- ☐ A. No. It flags and routes to a clinician
- ☐ B. Yes, if BP is high for a week
- ☐ C. Yes, but only Zoe can

4 A nurse enters a suspicious value. The app should...

- ☐ A. Silently correct it
- ☐ B. Block it without a message
- ☐ C. Prompt a re-check or a comment

Quick check (2 of 2)

Pick one answer per question. Answers are on the next page. No score is kept.

5 A device pairing test fails before discharge. You...

- ☐ A. Mark it issued and fix it later
- ☐ B. Create a support task. It is not marked issued
- ☐ C. Skip the test

6 How are lab and ECG/EEG results joined to the patient?

- ☐ A. Specimen barcode or device session ID linked to the patient QR
- ☐ B. Staff type the patient's name
- ☐ C. Match by date and time

7 What do Control patients receive?

- ☐ A. Nothing
- ☐ B. The same app with fewer nudges
- ☐ C. Usual care + benchmark checks. No coaching, nudges or devices

8 The Noreon doctor and the GP disagree on part of a plan. What happens?

- ☐ A. The Noreon doctor's version is published
- ☐ B. That part stays pending until the accountable clinician resolves it
- ☐ C. The plan is published and fixed later

Quiz answers

Miss one? Tap Review to jump to the page that explains it.

1	B	After consent and baseline, before any Noreon intervention	Baseline stays independent of the intervention.	Review
2	C	An opaque, short-lived token	A screenshot of it must reveal nothing.	Review
3	A	No. It flags and routes to a clinician	Only with a specific protocol and legal approval, which is not assumed.	Review
4	C	Prompt a re-check or a comment	No silent blocking and no silent fixing.	Review
5	B	Create a support task. It is not marked issued	Unless a documented non-connected fallback is chosen.	Review
6	A	Specimen barcode or device session ID linked to the patient QR	Never by typing a name.	Review
7	C	Usual care + benchmark checks. No coaching, nudges or devices	Clinically important findings still follow normal care.	Review
8	B	That part stays pending until the accountable clinician resolves it	Both proposals and the final decision go in the audit trail.	Review

Glossary: plain-language terms

ArMed. The polyclinic's existing medical record system. In this pilot data moves in and out by hand.

Zoe. Noreon's patient assistant. Explains the approved plan and supports daily actions.

Anamnesis. The patient's structured health history, answered in the app.

Pilot / Control. Pilot gets the full Noreon program. Control gets usual care plus matched measurements.

Benchmark. The Control group's outcomes, used to judge Noreon's extra effect.

Crossover. A Control patient starts the Noreon intervention. Recorded explicitly.

Contamination. Control patients being influenced by the intervention, which weakens the comparison.

Provenance. Where a data point came from: who or what, and when.

Crosswalk. A table linking one thing's IDs across systems (Noreon, lab, device).

Station QR. A short-lived code that opens the patient's session at each station.

FYI / Action required. FYI: no click needed. Action required: a clinician must decide.

Care-plan version. One approved, dated snapshot of targets, medicines, tests and goals.

Red flag. A symptom, reading or event that must go to a clinician by the approved protocol.

SLA. The target response time for an alert or task. Set by the medical team.

Heads-up for the document owners

These are my own observations while building this guide. They are not part of the source blueprint.

INCONSISTENCIES SPOTTED

- ☐ Stage 8 has two “Step 6” entries (manual ArMed update, patient notification). This guide numbers them 6 and 7.
- ☐ §2.1 still says “baseline visit” and mentions an activation link at booking. The updated flow says “selection visit”, and activation belongs to Pilot onboarding.
- ☐ The front-desk journey bar (§7.3) skips selection and cohort assignment.
- ☐ Consent appears twice: evaluation consent (Stage 2) and the consent package (Stage 3). Say which scopes belong where.
- ☐ §9 lists Control states twice and has overlapping cohort lists. ELIGIBLE_NOT_ASSIGNED is never explained.
- ☐ ECG vs EEG is open in §1.3, but “ECG/EEG” still runs through Stage 5, §7.6 and §13.

NOT YET ANSWERED

- ☐ Primary outcome and the follow-up windows for Pilot vs Control.
- ☐ Sample size per group and the allocation ratio.
- ☐ Who collects Control benchmark data (§13 says manual extraction), and what consent covers it.
- ☐ How Control patients’ clinically important findings are routed in practice.
- ☐ How contamination (Control exposed to Noreon) will be prevented and measured.

My notes

Type here, then save a copy of the PDF to keep your notes.

Questions for the team

My to-dos

Things I still do not get